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R SHAHIRA BEGAM

Female, 50 years

Date: 26 September, 2026

Reports: Health Check @ Home - 33 tests

Patient Name : MS. R SHAHIRA BEGAM
Age / Gender : 50 years / Female
Patient ID :196739
Referral : Dr. SELF
Sample Type : Edta Wb

Source : Doctor C Banglore
Collection Time : Sep 26, 2026, 09:32 a.m.
Receiving Time : Sep 27, 2026, 10:18 a.m.
Reporting Time : Sep 27, 2026, 11:06 a.m.
Sample ID : 7165141

HEALTH PACKAGE - 33 TESTS
HAEMATOLOGY

Test Description	Value(s)	Biological Reference Range	Unit(s)	Methodology
Complete Blood Picture (CBP/CBC)				
Haemoglobin	**7.0**	12.0 - 15.0	gm/dL	Photometric Measurement
Total RBC Count	**5.04**	3.8 - 4.8	mil/cu.mm	Coulter Principle
Total WBC Count	6350	4000-10000	cell/cu.mm	Coulter Principle
Platelet Count	**516**	150 - 410	10^3/ul	Coulter Principle
Red Cell Distribution Width	**21.0**	11.6 - 14.0	%	Derived from RBC Histogram
Hematocrit	**27.0**	36 - 46	%	Calculated
Mean Cell Volume (MCV)	**53.5**	83 - 101	fL	Derived from RBC Histogram
Mean Cell Haemoglobin (MCH)	**14.0**	27 - 32	pg	Calculated
Mean Corpuscular Hb Concn. (MCHC)	**25.9**	31.5 - 34.5	gm/dL	Calculated
Neutrophils	62.8	40 - 80	%	Optical Impedance
Lymphocytes	26.8	20 - 40	%	Optical Impedance
Monocytes	6.7	2 - 10	%	Optical Impedance
Eosinophils	3.6	1 - 6	%	Optical Impedance
Basophils	0.1	0-2	%	Optical Impedance
DC Sum	100.00			
Absolute Neutrophil Count	3.99	2.0 - 7.0	* 10^9/L	Calculated
Absolute Lymphocyte Count	1.70	1-3	* 10^9/L	Calculated
Absolute Monocyte Count	0.43	0.2-1.0	* 10^9/L	Calculated
Absolute Eosinophil Count	0.23	0.0-0.5	* 10^9/L	Calculated
Absolute Basophils Count	**0.01**	1-2	* 10^9/L	Calculated
RBC	Microcytic Hypochromic with Anisopoikilocytosis			
WBC	Normal in Total Count and Differential			
Platelets	Thrombocytosis			

Reference

Fully automated haematology analyzer (Beckman DXH-560) (Colorimetry, Electrical Impedance, VCS Technology, Leishman's Stain and Microscopy). **Reference :** Dacie and Lewis Practical Hematology,12th Edition

END OF REPORT

Dr. Karthik LR
Consultant Pathologist
Reg No 107509

Patient Name : MS. R SHAHIRA BEGAM
Age / Gender : 50 years / Female
Patient ID :196739
Referral : Dr. SELF
Sample Type : Serum

Source : Doctor C Banglore
Collection Time : Sep 26, 2026, 09:32 a.m.
Receiving Time : Sep 27, 2026, 10:10 a.m.
Reporting Time : Sep 27, 2026, 11:38 a.m.
Sample ID : 7165143

HEALTH PACKAGE - 33 TESTS
BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Range	Unit(s)	Methodology
Renal Function Test				
Blood Urea Nitrogen	7.34	4 - 18 :New Born/Chid 6 - 20 : Adult 8 - 23 : > 60 years	mg/dL	Calculated
Urea - Serum	**15.7**	Adult 17-43 New Born 8.4 -25.8 Infant/Child 10.8-38.4	mg/dl	UV-GLDH
Creatinine -serum	0.74	0.51 - 0.95 Neonate -0.31 - 0.98 Infant - 0.16 - 0.39 Child -0.262 - 0.77	mg/dL	Jafee Method-A
Urea Creatinine Ratio	21.22	Elevated ratio : >100.1 Reduced ratio : <40.1	mg/mg	Calculated
Uric Acid	3.9	2.6 – 6.0	mg/dL	Enzymatic Colour Test

Interpretation

- Creatinine: Muscles produce creatinine, a waste product, from creatine phosphate, a substance that stores a lot of energy. Unlike urea, the amount of creatinine generated is constant and mostly depends on muscle mass. Age, gender, race, muscularity, exercise, pregnancy, and several other physiological characteristics can all have an impact on serum creatinine levels.
- Decreased serum Creatinine is associated with increasing Age and poor muscle mass, such as muscular atrophy. Both acute and chronic renal disease and blockage are associated with elevated blood creatinine levels.
- Creatinine is not an appropriate indicator for identifying kidney disease in its early stages since an increase in blood creatinine is only seen when there is significant nephron damage. High Urea, Uric Acid, and Blood Urea Nitrogen (BUN) could indicate poor renal function, in addition to other etiologies

Reference :

Beckman Series Kit Insert

END OF REPORT

Dr. Karthik LR
Consultant Pathologist
Reg No 107509

Patient Name : MS. R SHAHIRA BEGAM
Age / Gender : 50 years / Female
Patient ID :196739
Referral : Dr. SELF
Sample Type : Serum

Source : Doctor C Banglore
Collection Time : Sep 26, 2026, 09:32 a.m.
Receiving Time : Sep 27, 2026, 10:10 a.m.
Reporting Time : Sep 27, 2026, 11:38 a.m.
Sample ID : 7165143

HEALTH PACKAGE - 33 TESTS
BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Range	Unit(s)	Methodology
Lipid Profile				
Cholesterol-Total	133.2	Desirable: <= 200 Borderline High: 201-239 High: > 239	mg/dL	CHE-CHO-POD
Cholesterol-HDL Direct	47.7	High Risk: < 40 Optimal: 40 - 60 Low Risk: > 60	mg/dL	Enzymatic
LDL Cholesterol	75.28	Optimal: < 100 Near / Above optimal: 100 - 129 Borderline high: 130 - 159 High: 160 - 189 Very High: >= 190	mg/dL	Calculated
Triglycerides	51.1	Normal: < 150 Borderline High: 150 - 199 High: 200 - 499 Very High: >= 500	mg/dL	GK-GPO-POD
Non - HDL Cholesterol	85.50	Desirable: < 130 Borderline High: 130 - 159 High: 160 - 189 Very High: >= 190	mg/dL	calculated
VLDL Cholesterol	10.22	10 - 30	mg/dL	calculated
CHOL/HDL RATIO	2.79	0.0 - 5.0	ratio	calculated
LDL/HDL RATIO	1.58	0.0 - 3.5	ratio	calculated
HDL/LDL RATIO	0.63	0.0 - 3.5	ratio	calculated

Interpretation

Interpretation:

• For non-fasting samples, the biological reference interval remains the same for all parameters, except for triglyceride as cholesterol (HDL, LDL, total), which changes only by a small amount in the non-fasting state; the recommended desired value for triglycerides is 200 mg/dl, are recommended to perform a follow-up fasting lipid panel in 2 to 4 weeks.

• As per the consensus of the Lipid Association of India, Non-HDL cholesterol and LDL cholesterol can be used as targets to monitor the effectiveness of lipid-lowering therapy.

Associated tests: Apolipoproteins A1, Apolipoproteins B, Apolipoprotein B/A1 Ratio, Lipoprotein(a)

Reference :

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Referral : Dr. SELF
Sample Type : Serum

Source : Doctor C Banglore
Collection Time : Sep 26, 2026, 09:32 a.m.
Receiving Time : Sep 27, 2026, 10:10 a.m.
Reporting Time : Sep 27, 2026, 11:38 a.m.
Sample ID : 7165143

HEALTH PACKAGE - 33 TESTS
BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Range	Unit(s)	Methodology

Tietz textbook of Clinical Chemistry, Third Edition. Carl A. Burtis and Edward R.Ashwood, eds. Philadelphia, PA: WB saunders.

Beckman Series Kit insert

END OF REPORT

Dr. Karthik LR
Consultant Pathologist
Reg No 107509

Patient Name : MS. R SHAHIRA BEGAM
Age / Gender : 50 years / Female
Patient ID :196739
Referral : Dr. SELF
Sample Type : Fluoride - F

Source : Doctor C Banglore
Collection Time : Sep 26, 2026, 09:32 a.m.
Receiving Time : Sep 27, 2026, 10:31 a.m.
Reporting Time : Sep 27, 2026, 12:17 p.m.
Sample ID : 7165139

HEALTH PACKAGE - 33 TESTS
BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Range	Unit(s)	Methodology
Glucose - Fasting				
Glucose fasting	75.0	Normal: 70 - 106 Impaired Tolerance: 101-125 Diabetes mellitus: >= 126	mg/dL	Hexikinase G6P-DH

Interpretation

A fasting blood glucose test is clinically significant because it is the most common method to screen for prediabetes and diabetes, as it measures blood sugar levels after a period of fasting, providing a reliable indicator of how well your body regulates glucose when not actively consuming food; high fasting blood glucose levels can indicate an increased risk of developing diabetes or related complications, even if symptoms aren't present.

Reference:

Beckman Series Kit insert

END OF REPORT

Dr. Karthik LR
Consultant Pathologist
Reg No 107509

Patient Name : MS. R SHAHIRA BEGAM
Age / Gender : 50 years / Female
Patient ID :196739
Referral : Dr. SELF
Sample Type : Serum

Source : Doctor C Banglore
Collection Time : Sep 26, 2026, 09:32 a.m.
Receiving Time : Sep 27, 2026, 10:10 a.m.
Reporting Time : Sep 27, 2026, 01:51 p.m.
Sample ID : 7165143

HEALTH PACKAGE - 33 TESTS
BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Range	Unit(s)	Methodology
Thyroid Stimulating Hormone (TSH)				
TSH(THYROID STIMULATING HORMONE)	2.765	Adult / Female 0.38 to 5.33 Pregnant Females 1st Trimester 0.05 to 3.70 2nd Trimester 0.31 to 4.35 3rd Trimester 0.41 to 5.18	µIU/mL	CLIA

Interpretation:

1.T3 &T4 values may be altered due to changes in serum proteins,pregnancy,drugs,nephrosis etc.In such cases Free T3 and Free T4 may give more appropriate thyroid status.T3 levels fluctuate rapidly to stress and non thyroid illness.
2.TSH values may be transiently altered in fever,severe infections,liver disease,renal and heart failure,severe burns,trauma and surgery.
3.Drugs that decrease TSH values include L-DOPA,Glucocorticoids,Heparin.Drugs that increase TSH include-Iodine,Lithium,Amiodarone.

Refrence : Beckman Series Kit Insert

END OF REPORT

Dr. Karthik LR
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Reg No 107509

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